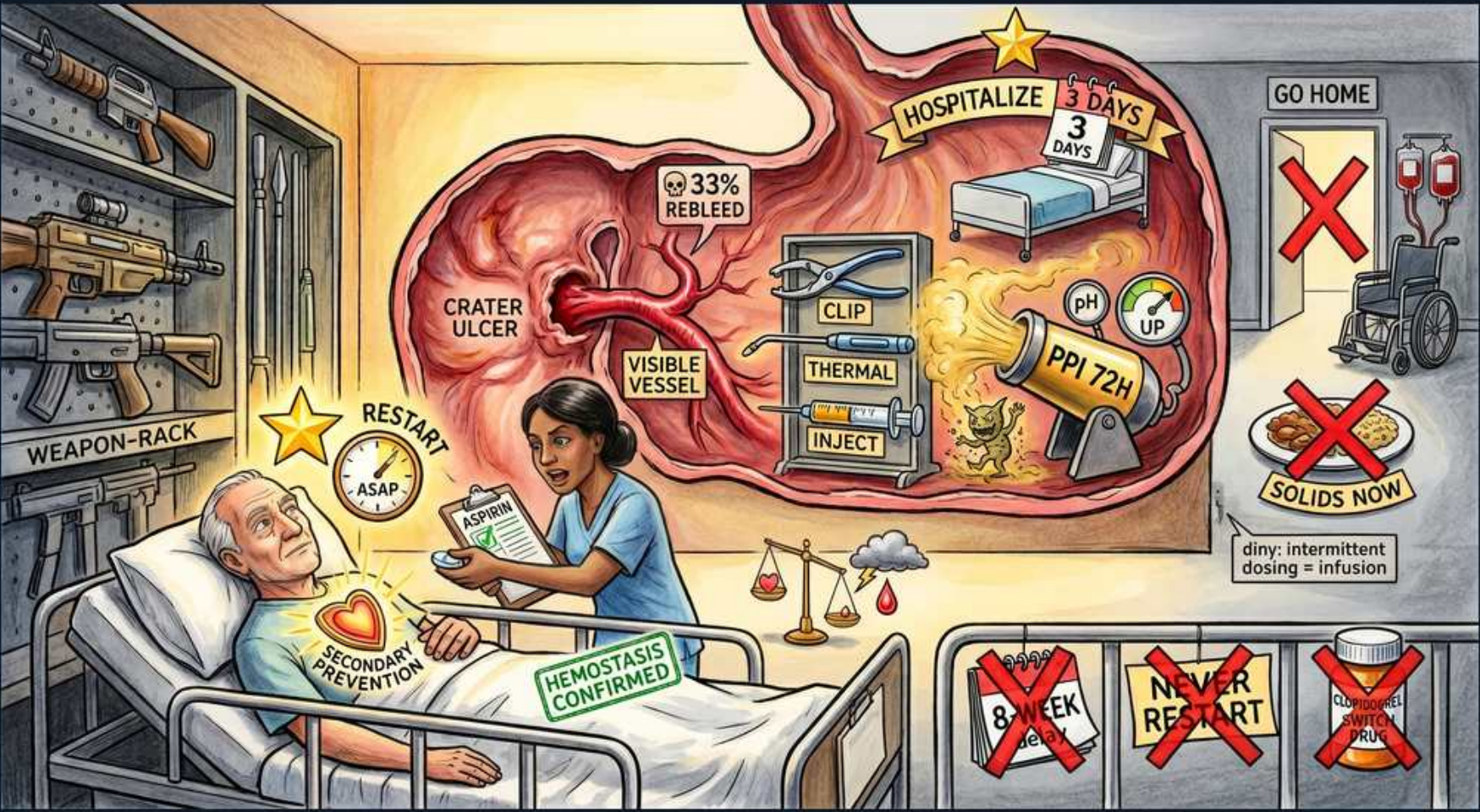


GI Bleed — Peptic Ulcer & Aspirin



1. Crater ulcer

WHAT YOU SEE

Large red mucosal crater labeled 'CRATER ULCER' in the stomach.

WHAT IT ENCODES

Peptic ulcer = the most common cause of upper GI bleeding. The endoscopic appearance (Forrest classification) of the ulcer base predicts rebleeding and dictates whether endoscopic therapy is needed.

2. Visible vessel ~33% rebleed

WHAT YOU SEE

Banner '33% REBLED' with a star pointing to a 'VISIBLE VESSEL' in the ulcer base.

WHAT IT ENCODES

High-risk Forrest stigmata: active spurting (Ia) ~ up to 90% rebleed, non-bleeding visible vessel (IIa) ~33-50%, adherent clot (IIb) ~20-30%. These require endoscopic hemostasis. Clean-based ulcers (III) and flat spots (IIc) are low risk -> no therapy, can feed early.

3. Combine TWO modalities

WHAT YOU SEE

Toolbox showing 'CLIP', 'THERMAL', 'INJECT' stacked together.

WHAT IT ENCODES

Endoscopic hemostasis = TWO modalities combined: injection (dilute epinephrine 1:10,000) PLUS a second method (thermal coaptive coagulation OR mechanical clips). Dual therapy lowers rebleeding versus any single method. Epinephrine is only the first half.

4. Epinephrine alone (RED X concept)

WHAT YOU SEE

'INJECT' panel highlighted alone in the toolbox.

WHAT IT ENCODES

Epinephrine injection ALONE is insufficient; it gives transient vasoconstriction/tamponade but high rebleed rates. It must be paired with a second modality (thermal or clip). Choosing epinephrine monotherapy is a classic wrong answer.

BOARD TRAP

'epinephrine injection alone' -> wrong; monotherapy rebleeds; must add a second modality (thermal or clip).

5. High-dose IV PPI 72h

WHAT YOU SEE

Cannon labeled 'PPI 72H' with a 'pH UP' gauge above it.

WHAT IT ENCODES

After endoscopic therapy of a high-risk ulcer, give high-dose IV PPI for 72h (e.g., pantoprazole 80 mg bolus then 8 mg/h infusion, or intermittent high-dose). Raising intragastric pH >6 stabilizes the clot and reduces rebleeding, surgery and mortality. Intermittent high-dose dosing is non-inferior to continuous infusion.

6. Hospitalize >=3 days (gold star)

WHAT YOU SEE

Gold-star banner 'HOSPITALIZE 3 DAYS' over a hospital bed icon.

WHAT IT ENCODES

High-risk ulcers (active bleeding, visible vessel, clot) rebleed mostly within 72h, so admit and observe for at least 3 days. Low-risk clean-based ulcers can be fed and discharged early. Risk-stratify with Forrest + clinical scores (Glasgow-Blatchford 0-1 = outpatient).

7. Restart aspirin ASAP (gold star)

WHAT YOU SEE

Gold-star 'RESTART ASAP' over a nurse handing the patient an 'ASPIRIN' card.

WHAT IT ENCODES

If aspirin is for SECONDARY cardiovascular prevention, restart it as soon as hemostasis is secured, usually within 1-3 days (continue PPI). The cardiovascular/thrombotic risk of withholding aspirin outweighs the small rebleed risk. Do not stop it long-term.

8. Secondary prevention indication

WHAT YOU SEE

Label 'SECONDARY PREVENTION' across the patient's gown.

WHAT IT ENCODES

The restart rule applies to true secondary prevention (prior MI, stroke, stent, established CAD). For pure primary prevention with bleeding, aspirin can often be stopped permanently. Distinguish the indication before deciding.

9. Hemostasis confirmed

WHAT YOU SEE

Banner 'HEMOSTASIS CONFIRMED' near the patient.

WHAT IT ENCODES

Aspirin resumption is gated on confirmed endoscopic hemostasis. Once the vessel is treated and not actively bleeding, the protective benefit of restarting outweighs rebleed risk; pair with continued PPI.

10. Go home / discharge now (RED X)

WHAT YOU SEE

Door labeled 'GO HOME' with a wheelchair and blood bags, crossed by a red X.

WHAT IT ENCODES

Discharging a high-risk ulcer immediately is wrong; they need >=72h observation for rebleed. Only low-risk (clean base, Glasgow-Blatchford <=1) patients may go home early.

BOARD TRAP

'discharge home immediately' for a high-risk ulcer -> wrong; rebleeding peaks in 72h, admit >=3 days.

11. Solids now (RED X)

WHAT YOU SEE

Plate of solid food with a red X labeled 'SOLIDS NOW'.

WHAT IT ENCODES

Allowing solids immediately after high-risk ulcer therapy is wrong; keep NPO/clear-liquids initially in case of rebleed needing re-scope or surgery. Early feeding is appropriate only for low-risk clean-based ulcers.

BOARD TRAP

'resume solid food immediately' in a high-risk ulcer -> wrong; advance diet slowly given rebleed/re-scope risk.

12. Delay aspirin 8 weeks (RED X)

WHAT YOU SEE

Red-X card '8 WEEKS' at the bottom.

WHAT IT ENCODES

Waiting 8 weeks to restart aspirin is wrong for secondary prevention: prolonged withdrawal sharply raises MI/stroke/stent-thrombosis risk. Restart within days once hemostasis is secured, under PPI cover.

BOARD TRAP

'delay aspirin 8 weeks' -> wrong; for secondary prevention restart within days of secured hemostasis.

13. Never restart (RED X)

WHAT YOU SEE

Red-X card 'NEVER RESTART'.

WHAT IT ENCODES

Permanently stopping aspirin after a GI bleed in a secondary-prevention patient roughly doubles cardiovascular mortality. The correct answer is restart, not abandon.

BOARD TRAP

'never restart aspirin' -> wrong; permanent withdrawal in secondary prevention increases cardiovascular death.

14. Switch to clopidogrel (RED X)

WHAT YOU SEE

Red-X pill card (clopidogrel) at far bottom right.

WHAT IT ENCODES

Switching aspirin to clopidogrel after a GI bleed is wrong; clopidogrel monotherapy causes as much or more GI bleeding and is not protective. Correct strategy is aspirin + PPI, not a switch.

BOARD TRAP

'switch aspirin to clopidogrel' -> wrong; clopidogrel bleeds as much; keep aspirin + PPI.
